

Stridor

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Stridor is a gasping sound during inhalation resulting from a partial blockage of the throat (pharynx), voice box (larynx), or windpipe (trachea).

Stridor is usually loud enough to be heard at some distance. The sound is caused by turbulent airflow through a narrowed upper airway.

In **children**, the cause may be:

- [Croup](#)
- An inhaled foreign object
- An [infection of the epiglottis](#) (rare)

In **adults**, the cause may be:

- A tumor
- An [abscess](#)
- Swelling (edema) in the upper airway
- [Vocal cord paralysis or dysfunction](#)
- An inhaled foreign object (occasionally)
- An [infection of the epiglottis](#)

Stridor that causes [shortness of breath](#) when the person is at rest is a medical emergency.

[Stridor](#)

Stridor and severe shortness of breath can be due to an allergic reaction (anaphylaxis) with swelling of the airway caused by certain foods, medications, insect stings, or latex.



Tumors of the head and neck such as [laryngeal cancer](#) or an abscess can cause stridor if they partially obstruct the upper airway.

When both vocal cords are paralyzed, the space between the paralyzed cords is very small, and the airway is inadequate so that even moderate exertion causes difficulty in breathing and a harsh, high-pitched sound (stridor) with each breath.

Brief episodes of stridor can result when a person inhales a food particle or bit of water into the lungs, causing contraction of the voice box (larynx). If an inhaled foreign object is a likely cause and the person is outside of a hospital, another person, if trained, can sometimes help expel the foreign object. For example, in an adult, the other person may use abdominal thrusts (previously called the [Heimlich maneuver](#)).

If a person has stridor and is in a hospital or emergency department, a tube may be inserted through the person's mouth or nose (tracheal intubation) or through a small surgical incision directly into the trachea (tracheostomy) to allow air to get past the blockage and prevent suffocation.

The cause usually becomes clear during tracheal intubation, when the doctor can see the upper airway directly. If tracheal intubation is not done, the diagnosis is usually determined by inserting a flexible viewing tube through the nose and upper airway (a procedure called nasopharyngeal laryngoscopy).

Once the cause is identified and the person's breathing is stabilized, treatment is focused on the disorder that caused stridor.

